

The concussion checklist you run before you run again.

A hit to the head is not the injury you manage. The next 72 hours, the fourteen days after them, and the return decision at the end are. Work this page top to bottom. Nothing here is skippable because the athlete says they feel fine.

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For athletes, parents, coaches and athletic trainers

Fillable PDF. Tick as you go. Educational resource, not a diagnosis.

Before anything else: if any of these appear, get the athlete seen now

These are signs of a bleed, a swelling brain or a neck injury, not a routine concussion. They can start hours after the hit. Get the athlete in front of a provider as soon as you can, today rather than tomorrow, and do not wait to see whether it settles on its own. Do not drive them yourself if you suspect a neck injury.

- ◆ A headache that keeps getting worse, or a sudden severe headache
- ◆ Vomiting more than once
- ◆ Neck pain or tenderness after the hit. Keep them still.
- ◆ A seizure or convulsion
- ◆ One pupil bigger than the other
- ◆ Drowsiness they cannot be roused from, or will not stay awake
- ◆ Slurred speech, or speech that stops making sense
- ◆ Weakness, numbness or burning in the arms or legs
- ◆ Confusion that is increasing, agitation or combative behaviour
- ◆ Cannot recognise people or places, or repeats the same question
- ◆ Double vision that does not clear
- ◆ Loss of consciousness of any length
- ◆ Loss of balance they cannot correct, or cannot walk
- ◆ Anything that is getting worse rather than settling

Lower the threshold to zero if the athlete takes blood thinners, has had a concussion in the last few months, is very young, or was hit in a high-speed collision. When you are deciding whether this is bad enough to get checked, that is your answer. A seizure, an athlete who cannot be roused, unequal pupils, repeated vomiting or neck pain after the hit mean an emergency department now rather than an appointment later.

Minutes to hours

Sideline and the rest of day one

Day one: remove, protect, record

The single most damaging decision in youth sport is letting a concussed athlete finish the game. A brain that has just been injured is least able to protect itself from the next hit. No test on a sideline is good enough to overrule that.

Do these today

☐ **Out of the game, out of practice, for the rest of the day.**

No same-day return, even if symptoms clear in ten minutes. If in doubt, sit them out.

☐ **Write down the mechanism and the time.**

What hit what, from which direction, at what speed, and the first three symptoms reported. Memory of this fades fast and clinicians ask for it.

☐ **Keep the video if there is any.**

A clip of the mechanism is more useful than any description of it.

☐ **An adult stays with them for the first few hours.**

You are watching for the red flags above, which often appear later than the hit.

☐ **No driving, no riding, no machinery.**

Reaction time and judgement are impaired before the athlete can feel that they are.

☐ **No alcohol and no recreational drugs.**

They mask deterioration and slow recovery.

☐ **Check with a clinician before any pain medication.**

Aspirin and anti-inflammatories are usually avoided in the first 24 to 48 hours because of bleeding risk. Do not start a medication routine on your own.

☐ **Book an evaluation within 24 to 48 hours.**

See a clinician who assesses concussions regularly. Ask for the neck, the balance system and the eyes to be looked at, not only the symptom list.

☐ **Tell the school, the coach and the athletic trainer today.**

Recovery runs through the classroom as much as the field. The people who set their workload need to know tonight, not next week.

☐ **Start the symptom log tonight.**

Use the tracker at the bottom of this page. Morning and evening. This is the data the return decision is built on.

The waking-every-hour myth

Once an athlete has been assessed and has no red flags, sleep is good for the injured brain. Waking them hourly through the night is not standard advice any more. Agree a check-in plan with the clinician who sees them, then let them rest.

24 to 48 hours

Relative rest,
not a dark
room

The first two days: rest relatively, then start moving

Two days of reduced load helps. A week of lying in a dark room does not, and tends to make the second week worse. The current standard is relative rest for 24 to 48 hours, then light activity introduced deliberately, even while some symptoms are still present.

The first 48 hours

☐ **Cut load, do not eliminate it.**

Short bursts of easy activity, conversation and light reading are fine. Stop the task before symptoms spike rather than after.

☐ **Screens in doses, not bans.**

Roughly ten to twenty minutes at a time with real breaks. Reduce brightness, raise text size, turn off autoplay motion.

☐ **Sleep at normal hours and protect the wake time.**

Naps before mid-afternoon only. A drifting sleep schedule is one of the most common reasons a recovery stalls in week two.

☐ **Eat and drink on a schedule.**

Athletes under-eat and under-drink when they are not training. Both amplify headache and fog.

☐ **Start light aerobic activity inside the first 48 hours if tolerated.**

Walking or an easy stationary bike. Symptoms may rise slightly during it. That is acceptable if the rise is mild and settles within about an hour.

☐ **Use the two-point rule.**

Rate symptoms out of ten before an activity. A rise of two points or less that settles quickly means the dose was right. More than that, or a rise that lasts, means you went too far. Repeat the previous level tomorrow.

☐ **No contact, no head-impact risk, no heading, no sparring.**

This applies to casual play and PE, not only to team training.

☐ **Re-read the red flags every evening for the first three nights.**

Deterioration is a late event. Knowing the list matters most on night two.

Symptoms are a dosing signal, not a verdict. You are looking for the largest amount of activity that does not make them meaningfully worse.

If the light and the screens are the problem, do not wait two weeks

Light sensitivity, words moving on the page, trouble tracking, or dizziness in busy rooms that are still there after a couple of days are worth looking at now. The sooner they are addressed, the faster we can get the visual system back online. Reach us at shahneurovision.com.

Days 3 to 14

Return to learn
comes first

Back to the classroom before back to the field

An athlete who cannot get through a school day without symptoms is not ready to be assessed for contact. Return to learn runs first, and it has its own steps. Most adults are through this in about two weeks. Children and adolescents commonly take up to four.

1	Daily activities at home that do not provoke symptoms Reading, short screen tasks, chores, walking. Build the tolerance before you add the environment.
2	School work at home Twenty to thirty minute blocks with breaks. Note which subject and which format cause trouble, especially reading and screens.
3	Part-time at school with accommodations Later start, shorter days, no tests, extra time, printed notes instead of the board, permission to leave a loud room.
4	Full days, then full workload Catch up on missed work gradually. A full day attended while symptomatic is not the same as a full day tolerated.

Habits that decide how week two goes

- ☐ **Keep light aerobic activity daily.**
Sub-symptom threshold exercise is treatment, not a reward for feeling better.
- ☐ **Expect mood changes and say so out loud.**
Irritability, flatness, anxiety and tearfulness are part of the injury for many athletes. Naming it early prevents it being read as attitude.
- ☐ **Watch the neck.**
The force that concussed the brain went through the neck first. Persistent headache at the base of the skull, or dizziness that moves with neck position, needs assessing.
- ☐ **Keep the athlete connected to the team.**
Attend sessions, keep the routine, keep the identity. Isolation lengthens recovery.
- ☐ **Log twice a day and look at the trend, not the day.**
A bad Tuesday inside a falling line is noise. A flat line across five days is information.
- ☐ **Book the follow-up before you need it.**
If the trend is flat at the ten-day mark, you want the appointment already in the diary.

The return ladder

Minimum 24
hours per step

Six steps back to competition

One step per day at the fastest. If symptoms rise by more than about two points out of ten and do not settle within an hour, that step was too much: rest a day and repeat the step below it. Medical clearance is required before contact, and it is a separate decision from feeling well.

1	Symptom-limited daily activity Walking, household and school activity kept below the level that provokes symptoms.
2	Light aerobic exercise Walking or stationary cycling at an easy pace. No resistance training. Goal is to raise heart rate gently.
3	Sport-specific exercise, no head-impact risk Running, skating, ball work alone. Moderate intensity, direction changes allowed, contact absolutely not.
4	Non-contact training drills Full team drills, passing, resistance training, harder cognitive load. This is the step where visual and decision-making problems usually surface.
5	Medical clearance required first Full contact practice Normal training in full. A clinician clears this step. Not the coach, not the parent, not the athlete.
6	Return to competition Unrestricted play. Keep logging for two weeks afterwards, especially after the first heavy session.

Do not let a good week become a shortcut

The ladder exists because symptoms at rest return to normal before the brain's tolerance for load does. An athlete can genuinely feel fine at step two and still fail at step four. Feeling fine is the entry requirement for the ladder, not a replacement for it.

Before you sign off on competition

- ☐ **Symptom-free through a full school or work day.**
Including the reading and screen load, not only the attendance.
- ☐ **Off any medication that was masking symptoms.**
A quiet symptom score on daily painkillers is not a clean score.
- ☐ **Tolerated full-intensity training with no symptom return.**
Including in noise, crowds and bright or changing light.
- ☐ **Balance, neck and eye movement checked, not assumed.**
These are the three systems most often still impaired when the symptom list reads zero.
- ☐ **Clearance documented by the clinician, with a date.**
Verbal clearance in a car park is not a record and it protects nobody.
- ☐ **A plan for what happens if symptoms come back in game one.**
Agreed in advance, with the coach, in writing.

The missed layer

Vision, balance and the visual system

When the symptom list is clear but the game still looks fast

Concussion regularly disturbs how the two eyes team, focus and move together, and how the eyes coordinate with the balance system. Those functions are not tested by a standard sight test and not captured by a symptom checklist. They are a common reason an athlete is cleared and still performs and feels wrong.

Reading falls apart

Losing the place, re-reading lines, words that swim or double after ten minutes.

Screens trigger headache

Pressure behind or between the eyes that builds with near work and eases when they stop.

Focus lags far to near

A beat of blur looking from the board to the notes, or from the field to the ball at their feet.

Busy places feel awful

Supermarkets, corridors between classes, crowds, scrolling backgrounds, car journeys.

Tracking is off

Late on a ball they used to read early, or misjudging where it will arrive.

Light and motion sensitivity

Squinting indoors, avoiding sunlight, nausea when the head and eyes move together.

Depth and spacing errors

Clipping door frames, misjudging a pass, standing slightly wrong to receive it.

Dizziness on head turns

Especially quick turns to scan, or looking up then back down at speed.

What to do about it

☐ Flag these symptoms specifically at the medical review.

Say "reading and screens", "dizzy in busy places", "late on the ball". Vague fog gets a vague plan.

☐ Ask that eye movements and the balance system are screened.

Convergence, focusing, tracking and vestibulo-ocular function. Screening tools flag a problem. They do not replace a full assessment.

☐ See a neurodevelopmental optometrist about the visual symptoms.

Also called a developmental or behavioural optometrist. This is the clinician who assesses how the eyes team, focus, track and work with the balance system, which a standard sight test does not cover. Book it if light sensitivity, reading trouble, tracking problems or dizziness are still there after a couple of days rather than waiting out the two-week mark.

☐ Do not sit on it.

The sooner these problems are addressed, especially when they have not cleared in a couple of days, the faster we can get the visual system back online.

☐ Ask about vestibular and cervical physiotherapy as well.

Dizziness, neck pain and visual symptoms travel together and are treated together.

☐ Re-test rather than re-guess.

Whatever was measured at the start should be measured again before the athlete is told the problem has resolved.

Still fighting the light, the page or the ball?

If light sensitivity, reading, focusing or tracking problems are still there, bring them to us. Shah NeuroVision assesses and treats exactly this, and the earlier it starts the less there is to undo. Book at shahneurovision.com or message [@drshah_neurovision](https://twitter.com/drshah_neurovision).

An honest line about what this does

Post-concussion vision care targets measurable visual deficits and the symptoms they produce, which is why reading, screen tolerance and dizziness are what improve first. Nobody has shown that it raises a batting average or any other sport statistic, and we will not tell you otherwise.

Beyond week two

Or week four
for a young athlete

If it is not clearing, stop waiting and start treating

Persisting symptoms are not a sign that the athlete needs more rest. They are a sign that something specific, usually in the neck, the balance system, the visual system, the autonomic response to exercise, sleep or mood, needs to be identified and treated directly.

Escalate to the right people

- ☐ **A concussion or sports medicine clinic for the overall plan.**
- ☐ **Vestibular and cervical physiotherapy for dizziness, balance and neck-driven headache.**
- ☐ **A neurodevelopmental optometrist for reading, focusing, tracking, light sensitivity and visual motion symptoms.**
This is our lane. Shah NeuroVision takes these referrals directly at shahneurovision.com, and the sooner it starts the faster the visual system comes back online.
- ☐ **A supervised exercise test to set a safe training heart rate.**
Guessing the intensity is the most common reason exercise-based recovery stalls.
- ☐ **Mental health support if mood, anxiety or sleep is the loudest symptom.**
This is a normal part of the injury and one of the most treatable parts of it.
- ☐ **Formal school accommodations in writing if the athlete is still limited.**

For parents and coaches: what athletes hide

- ☐ **Ask about function, not symptoms.**
"How long can you read before your head hurts" gets an honest answer. "Do you have a headache" gets "I'm fine".
- ☐ **Assume the report is under-stated by a good margin.**
Selection, scholarships and starting places all push athletes to minimise. So does not wanting to let the team down.
- ☐ **Remove the incentive to lie.**
Say out loud that the place is not at risk and that reporting early gets them back sooner, then behave that way.
- ☐ **Watch the things they cannot fake.**
Sleep, appetite, mood, screen tolerance, how long homework now takes, whether they are avoiding noise or light.
- ☐ **Never let a teammate, a coach or the athlete make the clearance call.**

Print this

Twice a day,
seven days

Symptom tracker

Rate each line from 0 (none) to 6 (severe), morning and evening. Add the column for a daily total. Take this to every appointment. Type straight into it, or print it and write. The shape of the line across a week is worth more than any single day's score.

Athlete	Sport and position	Date of injury	Clinician managing care

Symptom	Day 1	Day 2	Day 3	Day 4	Day 5	Day 6	Day 7
Headache or head pressure							
Dizziness or unsteadiness							
Nausea							
Sensitivity to light							
Sensitivity to noise							
Blurred or double vision							
Trouble reading or using screens							
Feeling slowed down or in a fog							
Trouble concentrating							
Trouble remembering							
Fatigue or low energy							
Irritability or short temper							
Sadness, flatness or anxiety							
Trouble falling or staying asleep							
Neck pain or stiffness							
Daily total							
Exercise done and how it felt							
Hours slept							

The symptoms end. The visual system doesn't always follow.

If an athlete has been cleared and still reads slowly, still hates crowded rooms, or still says the game looks faster than it used to, that is testable. Measure it, sharpen it, then send them back.

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Measure. Sharpen. Dominate.

This page is general education for athletes, families and coaching staff. It is not a diagnosis, not a treatment plan and not a substitute for assessment by a qualified clinician who has examined the athlete. Concussion management guidance changes, and an individual athlete's plan should always come from the clinician managing their care. If any red flag on this page appears, get the athlete in front of a provider straight away.